

OPPORTUNITY REVIEW

DIRECT-TO- EMPLOYER GTM STRATEGY

Exploring a Baptist Health DTE Health Plan





KEY BENEFIT ADMINISTRATORS

- Founded in 1979, headquartered in Indianapolis
- Largest Independently-Owned Full-Service Third-Party Administrator
- Specialize in Administration of Self-Funded Plans and Population Health Management
- Leader in the Direct-to-Employer Movement
- We Know and Understand the Market
- Experience Working with All Professional Teams: Agents, Networks, MGAs
- The Key Family of Companies is a Vertically Integrated group of benefit related organizations
 - 3,000+ Corporate Clients
 - 3 Million Members under Management
 - 700 Employees



Our Founding and Heritage:

46 Years of Striving for Honesty in Healthcare

Founded Key Benefit Administrators in 1979

After co-founding a life insurance agency in 1973 and Key Life Insurance in 1976, Larry Dust launched KBA in 1979 as a third-party benefits administrator focused on honest healthcare delivery.

Sold to Blue Cross/Blue Shield in 1985

Strategic partnership with BCBS of Indiana (then part of the Associated Group) provided capital and market access while maintaining operational independence.

Reacquired KBA in 1992

Larry bought the business back from BCBS in 1992, returning it to complete independence and patient-focused mission alignment.

Built the Largest Independent TPA in the U.S.

Under his leadership since the buyback, KBA has become the country's largest independently owned third-party administrator, managing millions of members across diverse markets.

Never Forgotten Who Gives Healthcare

KBA has never participated in reference-based pricing, does not take refunds, and delivers healthcare at 11% below market rates. Our commitment remains with providers and patients, not profit extraction.

Key Benefit Administrators

MARKET LEADER IN PRIVATE LABELED DTE PLANS

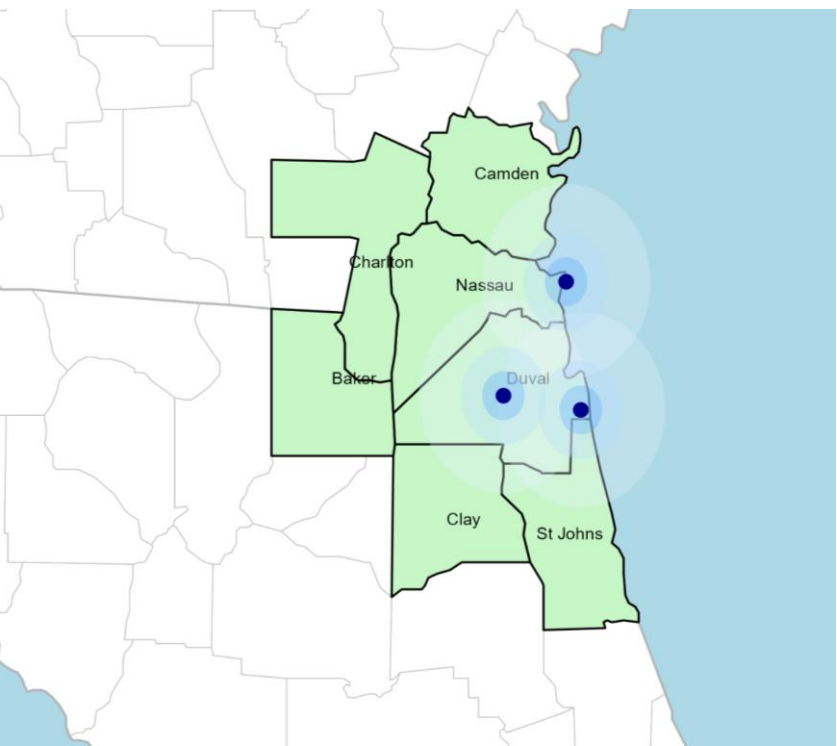


SIGNIFICANT OPPORTUNITY FOR PLAN GROWTH

Researched for the below highlighted service area

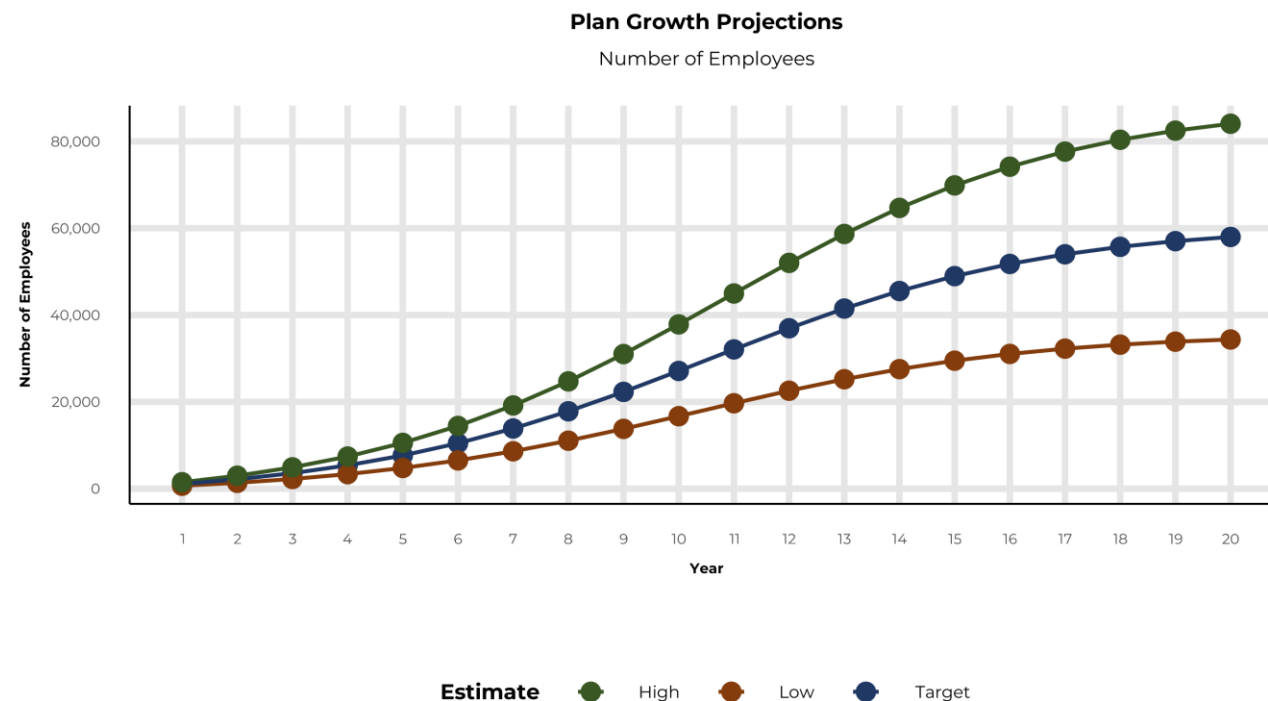
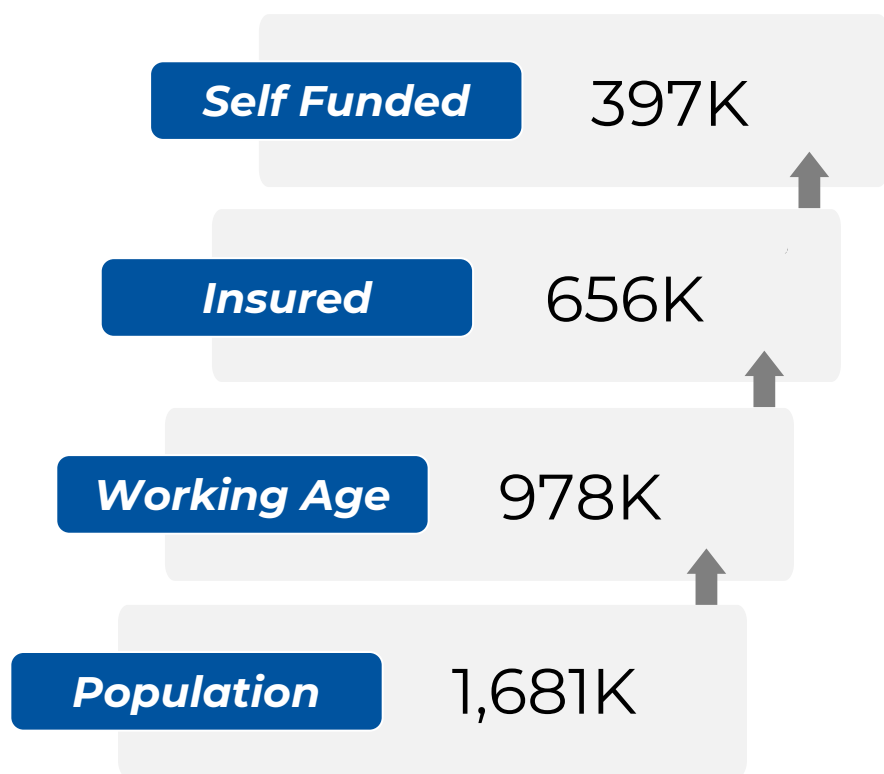
Target Market

1,500+ Employers ➤ 400,000+ Employees ➤ 600,000+ Members



	Employer Size Group (# of Employees)					
	50-100	101-200	201-400	401-1,000	1,001-3,000	3,000+
Commercial Employers With Headquarters In Catchment Area	911	352	192	141	56	29
Total Employees at Employers Headquartered in Catchment Area	68,325	52,800	57,600	98,700	112,000	319,000
Total Members with Employers Headquartered in Catchment Area	122,985	95,040	103,680	177,660	201,600	574,200

PLAN POTENTIAL GROWTH PATHS



Estimates includes the Baptist Health service area in Baker, Clay, Duval, Nassau, St. Johns, Camden, and Charlton Counties

GROWING POWERFUL, LASTING PROFITABILITY

Early Stage

Gaining Name Recognition, Early Growth

New Revenue:

\$21.36M

New Profit:

\$5.73M

Growth Stage

Strong Reputation, Strong Growth

New Revenue:

\$129.89M

New Profit:

\$35.81M

Mature Stage

High Profitability, Market Saturation

New Revenue:

\$269.11M

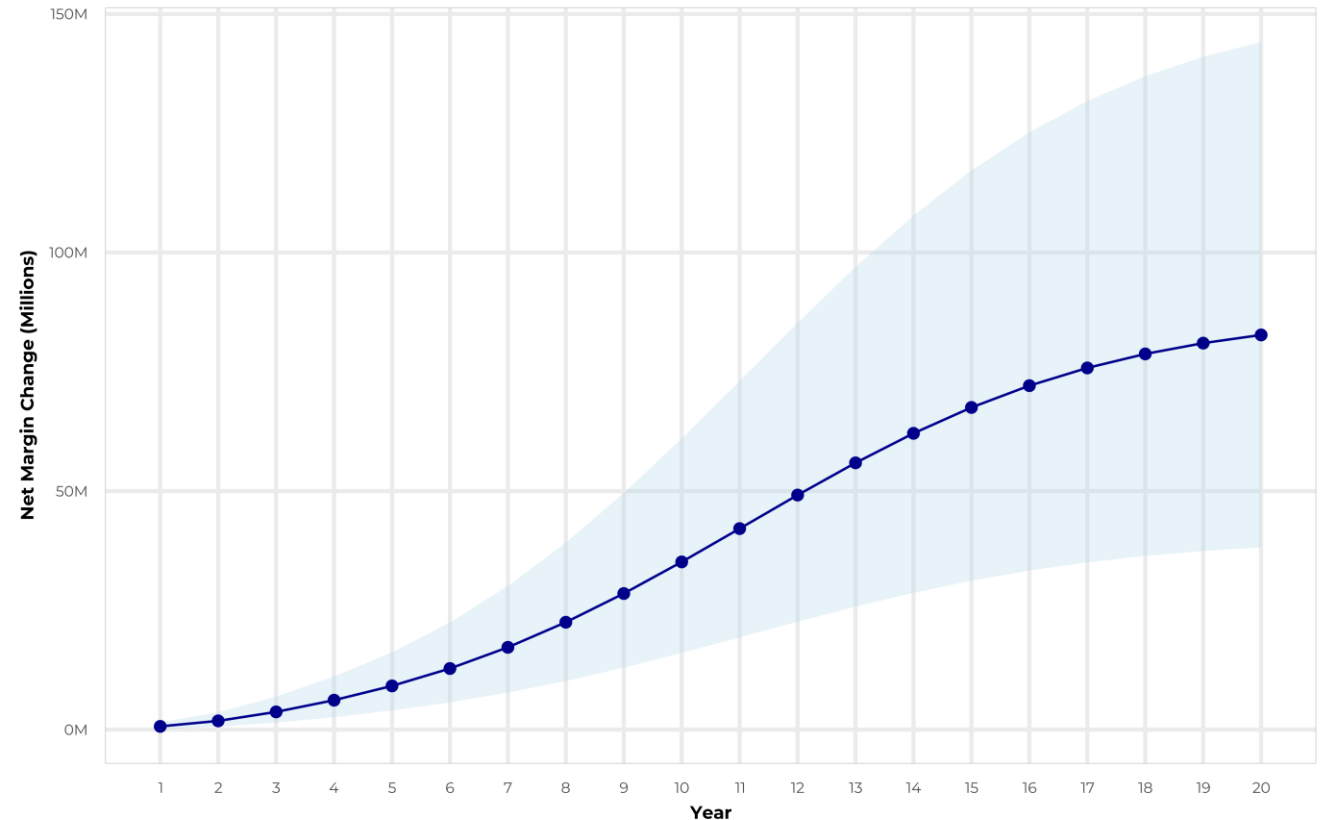
New Profit:

\$74.29M

Net Margin Change Over Years

Variation by Member Growth With Target Estimate

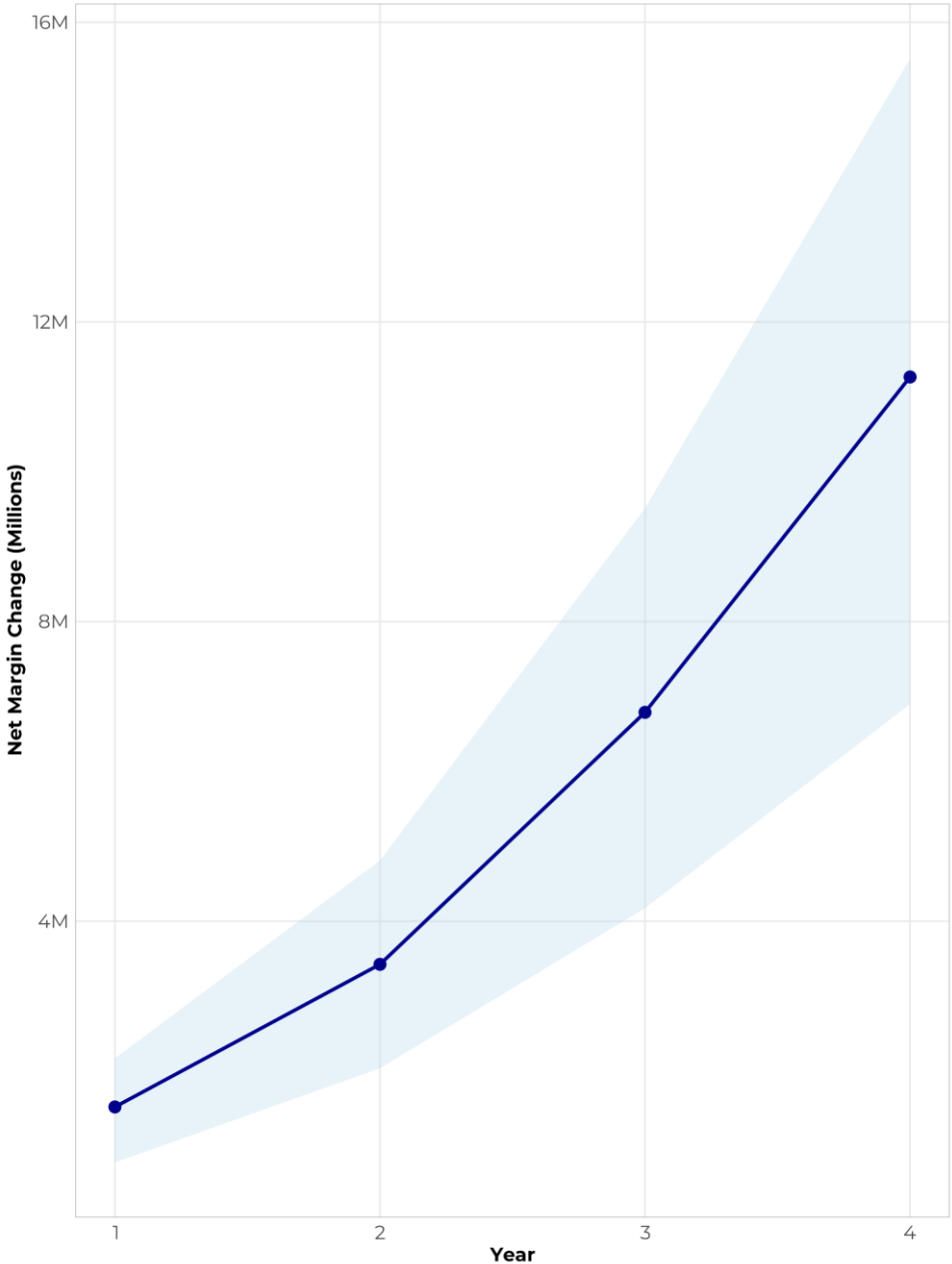
of 45%+ Steerage and a Commercial Margin of 22.4%



FOUR-YEAR PROJECTIONS

Net Margin Change Over Years
Variation by Member Growth With Target Estimate

of 45%+ Steerage and a Commercial Margin of 22.4%



Target Member Growth				
Metric	Year 1	Year 2	Year 3	Year 4
Number of Members	2,000	3,900	6,400	9,700
Incremental Margin	1,825.2K	3,556.3K	6,890.9K	11,328.7K
Network Access Fee	24.0K	46.8K	76.8K	116.4K
Implementation Fee (One-Time)	-150.0K	0.0K	0.0K	0.0K
Sales and Marketing Costs	-180.0K	-180.0K	-180.0K	-180.0K
Net Margin Change	1,519.2K	3,423.1K	6,787.7K	11,265.1K

High Member Growth				
Metric	Year 1	Year 2	Year 3	Year 4
Number of Members	2,700	5,400	8,900	13,300
Incremental Margin	2,464.1K	4,924.1K	9,582.7K	15,533.1K
Network Access Fee	32.4K	64.8K	106.8K	159.6K
Implementation Fee (One-Time)	-150.0K	0.0K	0.0K	0.0K
Sales and Marketing Costs	-180.0K	-180.0K	-180.0K	-180.0K
Net Margin Change	2,166.5K	4,808.9K	9,509.5K	15,512.7K

Low Member Growth				
Metric	Year 1	Year 2	Year 3	Year 4
Number of Members	1,200	2,400	4,000	6,000
Incremental Margin	1,095.1K	2,188.5K	4,306.8K	7,007.4K
Network Access Fee	14.4K	28.8K	48.0K	72.0K
Implementation Fee (One-Time)	-150.0K	0.0K	0.0K	0.0K
Sales and Marketing Costs	-180.0K	-180.0K	-180.0K	-180.0K
Net Margin Change	779.5K	2,037.3K	4,174.8K	6,899.4K

DIRECT-TO-EMPLOYER STRATEGY:

ONLY AVAILABLE TO *PREMIER* HEALTH SYSTEMS



STRONG Brand Quality

EXTENSIVE Patient Access

AFFORDABLE Value Pricing

EXCELLENT Care Quality

BAPTIST HEALTH IS PATIENT RECOMMENDED

Health System	HCAHPS Recommendation Percentile	CMS Stars Percentile	Mortality Percentile	Readmission Percentile	KBA Brand Quality Score
Mayo Clinic	98	100	81	63	77
Baptist Health (Jacksonville FL)	76	87	75	70	72
University of Florida Health	37	63	68	72	58
Ascension Health	40	58	80	60	58
HCA Healthcare	16	46	88	63	49

Based on quality measures and HCAHPS feedback, KBA estimates that Baptist Health has one of the strongest brands in Baker, Clay, Duval, Nassau, St. Johns, Camden, and Charlton Counties.

PROJECTED PLAN GROWTH

Year	Employees	Members	Percent of Members Choosing Baptist Health for Inpatient Care	Percent of Members Choosing Baptist Health for Outpatient Care
1	1,100	2,000	55%	45%
2	2,200	4,000	55%	45%
3	3,600	6,500	60%	50%
4	5,400	9,700	62%	53%
5	7,700	13,900	62%	54%
6	10,500	18,900	62%	55%
7	13,900	25,000	63%	56%
8	17,800	32,000	63%	56%
9	22,300	40,100	63%	57%
10	27,100	48,800	64%	57%
11	32,100	57,800	64%	58%
12	37,000	66,600	64%	58%
13	41,500	74,700	65%	59%
14	45,500	81,900	65%	59%
15	49,000	88,200	65%	60%
16	51,800	93,200	65%	60%
17	54,000	97,200	66%	61%
18	55,700	100,300	66%	61%
19	57,000	102,600	66%	61%
20	58,000	104,400	66%	61%

KBA currently estimates that Baptist Health has 30% inpatient market share, and 18% outpatient market compared to hospitals/clinics in the Baptist Health service area (see slide 8).

Based on previous DTE experience, KBA expects a significant bump in patient steerage to Baptist Health.

Additional steerage will occur from additional steerage patient benefits and the patient convenience card.

High estimates start nearly 10% higher than target with low estimates at 10% lower than target

YEAR 4 PROJECTED REVENUE AND PROFIT

Pre-DTE Cohort of 9,700 Employees	Inpatient	Outpatient	Professional	Total
Revenue†	\$ 9,825,129	\$ 10,791,230	\$ 5,039,412	\$ 25,655,771
Net Income‡	\$ 2,672,435	\$ 2,935,214	\$ 1,370,720	\$ 6,978,370

With DTE	Inpatient	Outpatient	Professional	Total
Revenue°	\$ 20,329,329	\$ 31,730,248	\$ 14,817,756	\$ 66,877,333
Net Income*	\$ 5,529,577	\$ 8,630,627	\$ 4,030,430	\$ 18,190,635

† Assuming proportional revenues from market shares of 30% (inpatient) and 18% (outpatient)

‡ Conservatively assuming a commercial margin of 27.2% (80% of the estimated commercial margin)

° Assuming 62% inpatient and 53% market steerage based on KBA narrow network DTE experience (see previous slide)

* Conservatively assuming a commercial margin of 27.2%, with a 10 percentage-point reduction to reflect competitive-edge rates and a 10 percentage-point increase due to lower billing costs and higher patient responsibility collections because of the member convenience card.

Total Increase in Net Income	\$ 11,212,265
Network Access Fee (Revenue) of \$1 PMPM	\$ 116,400
Total Profit Growth	\$ 11,328,665
Minus Implementation Fee (One-Time)	-\$ 0
Minus Sales and Marketing (Annual – Ongoing)	-\$ 180,000
Total	\$ 11,148,665

A PARTNER YOU CAN TRUST



**OUR OBJECTIVES
ARE ALIGNED**



**WE KNOW THE
MARKET**



**COMPLETELY
TRANSPARENT**



**WE HAVE THE
SOPHISTICATION
AND
INFRASTRUCTURE
TO EFFECTIVELY
COMPETE**



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